The Bolam Test

English tort law | Study guide | Professional skill, treatment and informed consent

1. A professional benchmark

Where the defendant is exercising or professing a special skill, the relevant comparison is ordinarily a reasonably competent practitioner exercising that skill. A medical treatment decision is not judged simply by what an untrained member of the public would do. The professional need not possess the highest possible expertise, but must meet the appropriate competent standard.

The skill must relate to the activity in issue. A historian's expertise does not automatically raise the standard for an unrelated driving incident. For professional work, identify the task and relevant field rather than attaching special liability to every action of a qualified person.

2. The Bolam principle

Bolam v Friern Hospital Management Committee [1957] 1 WLR 582 explains that a practitioner is not negligent merely because another group would choose differently, if the conduct follows a practice accepted as proper by a responsible body of appropriately skilled professional opinion.

That body need not represent a numerical majority. But the existence of someone willing to support the defendant is not an automatic defence. The opinion must be responsible and capable of logical support, as clarified in Bolitho.

3. Still an objective assessment

Choosing the relevant professional benchmark does not turn the test into the defendant's personal standard. The issue is not "Did this doctor do their personal best?" or "Did they honestly believe it was right?" The court assesses conduct against the applicable professional standard.

> Relevant skill → competent professional benchmark → responsible professional practice → logical scrutiny. Facts determine which benchmark applies; that does not make the test purely subjective.

The transcript's names are corrected to Bolam and Friern. A standard suited to professional work is not simply a universal "higher score" for every task; it concerns the particular skill being exercised.

Bolam: The Original Case

1. Facts: electroconvulsive therapy

Mr Bolam underwent **electroconvulsive therapy (ECT)**, a medical treatment using an electric current to induce a seizure. He suffered serious fractures during treatment. The negligence allegations concerned the absence of muscle-relaxant drugs, the extent of restraint and the lack of a warning about the fracture risk.

Medical witnesses disagreed about appropriate practice at the time, including the risks and benefits of relaxants and restraint. The case was therefore not only a claim about failing to give information; it also concerned how the treatment was administered.

2. Who won, and why?

The defendant hospital management committee won. McNair J directed the jury on the professional standard and the relevance of responsible medical opinion. The jury returned a verdict for the defendants. The injury itself did not establish negligent treatment.

A difference between two responsible schools of practice does not, by itself, make one of them negligent. Equally, professional status does not protect carelessness: the actual conduct must meet the required standard.

3. How to use the case now

Bolam remains important for clinical diagnosis and treatment, subject to judicial scrutiny under Bolitho. Its historical approach to warning patients must not be presented as today's general rule for informed consent. Montgomery requires a separate analysis of material risks and reasonable alternatives.

Invented example: two techniques are each supported by responsible, logically defensible opinion. Choosing one does not establish breach merely because another expert prefers the other. Failing to perform the chosen technique competently is a different question.

> Separate three questions: Was the chosen clinical approach professionally defensible? Was it carried out competently? Was the patient properly informed? One answer does not automatically resolve the others.

The example is invented. Bolam concerns medical practice in the 1950s. Its facts are legal history, not current clinical instructions for ECT. Do not infer current treatment guidance from the historical verdict.

Bolitho: A Logical Basis

1. Courts retain a role

In **Bolitho v City and Hackney Health Authority [1998] AC 232**, the House of Lords clarified that expert support is not conclusive merely because it is honestly given. The court must be satisfied that the professional opinion has a logical basis, including a defensible assessment of relevant risks and benefits.

This does not invite judges to replace medical expertise with their own preference. Where two competing professional opinions are both logically defensible, disliking one of them is not enough to reject it. The court described rejection of competent expert opinion as exceptional.

2. Facts and outcome

A young child suffered a catastrophic respiratory collapse. A doctor had negligently failed to attend when called. The further question was whether attendance would have led to intubation, and whether not intubating would itself have been negligent.

The health authority won the appeal. The expert view supporting non-intubation was not shown to be illogical. The claimant had not established that the admitted failure to attend caused the damage: a defensible decision not to intubate would not have prevented the collapse.

3. Why this matters for students

The case links breach and causation without merging them. Proving one negligent omission does not establish that it caused the injury. Conversely, a defendant cannot defeat causation simply by saying that a different negligent act would have occurred anyway.

Invented example: an expert supports omitting a simple precaution but cannot give a coherent reason for ignoring a known serious risk. The court may examine whether the opinion is genuinely responsible. “Another professional agrees with me” is not the complete legal test.

> Bolam gives weight to responsible professional practice. Bolitho requires that practice to withstand logical analysis. Neither principle guarantees a successful defence whenever an expert supports the defendant.

Bolitho was decided in 1997 and reported in [1998] AC 232. It is an added clarification to the transcript. The invented precaution example is not a further factual finding in that case.

Montgomery: Patient Choice

1. A different question: informed choice

Montgomery v Lanarkshire Health Board [2015] UKSC 11 concerned a woman with diabetes who was not told of the risk of shoulder dystocia during vaginal birth: the baby's shoulders becoming stuck. The complication occurred, and her son suffered serious injury.

Mrs Montgomery won in the Supreme Court. The doctor should have discussed the material risk and the option of caesarean delivery. The court also concluded that proper advice would have led her to choose a caesarean, avoiding the injury.

2. What is a material risk?

A doctor must take reasonable care to make a patient aware of material risks of recommended treatment and reasonable alternatives. A risk is material if a reasonable person in the patient's position would likely consider it significant, **or** the doctor knows or should reasonably know that this particular patient would likely consider it significant.

This is not just a question of what other doctors usually disclose. It protects the patient's decision. The assessment has a reasonable-patient aspect and a particular-patient aspect; it does not reduce to the doctor's subjective preference. Disclosure requires meaningful dialogue, not merely a signed form.

3. A later clarification: McCulloch

In **McCulloch v Forth Valley Health Board [2023] UKSC 26**, relatives argued that a drug option should have been discussed. **The health board won.** Identifying which alternatives are clinically reasonable uses the professional-practice test, subject to Bolitho; Montgomery governs discussion of material risks and reasonable options. Every conceivable treatment need not be offered.

> Clinical diagnosis/treatment: Bolam, qualified by Bolitho. Material-risk disclosure: Montgomery. Whether an alternative is clinically reasonable: professional-practice assessment clarified in McCulloch.

Correct name: Lanarkshire, not Lancashire. The Supreme Court cases arose in Scotland but clarify principles used in England and Wales. Narrow qualifications concerning serious harm from disclosure and urgent necessity do not permit withholding information simply to prevent a patient declining treatment.

Check Your Understanding

1. Why not compare a doctor with an untrained person?
Answer: The task requires professional skill. The appropriate benchmark is a reasonably competent practitioner exercising that relevant skill.

2. Does selecting a professional benchmark make the test subjective?
Answer: No. The facts identify the benchmark; the defendant's personal belief or personal best does not replace the objective standard.

3. Who won in Bolam?
Answer: The hospital management committee. The jury found for the defendants after directions on the professional standard; injury alone did not prove negligence.

4. Does one supporting expert guarantee a defence?
Answer: No. The conduct must reflect responsible professional practice capable of withstanding logical analysis.

5. What did Bolitho add, and who won?
Answer: It confirmed judicial scrutiny of the logical basis of expert opinion. The health authority won because the non-intubation opinion was defensible and causation was not established.

6. What is the Montgomery materiality test?
Answer: Would a reasonable person in the patient's position likely find the risk significant, or should the doctor know that this particular patient would? Either route can make a risk material.

7. Did Montgomery abolish Bolam for every medical issue?
Answer: No. It changed the approach to risk disclosure. Clinical treatment judgments remain distinct, and McCulloch addresses reasonable alternatives.

> Exam sequence: identify the task → distinguish clinical judgment from disclosure → choose the correct test → analyse evidence and reasons → conclude on breach → check causation and actionable damage.

Sources

Based on the supplied Law Academy transcript, with corrected names and legal distinctions. Added clarifications: Bolitho and McCulloch. Primary authorities: Bolam [1957] 1 WLR 582, 586-588; Bolitho [1998] AC 232. Montgomery [2015] UKSC 11, paras 87-90 and 103-105; McCulloch [2023] UKSC 26. Invented examples are marked.